

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	734
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	10/2011
		Review Date:	04/2028

HYPOTHERMIA	
ADULT	PEDIATRIC
Pulse and respirations may need to be checked for up to 60 seconds before beginning CPR. Bradycardia is not responsive to Atropine and is dependent on rewarming.	
Mild: 90° to 95°F (32° to 35°C) Moderate: 82° to 90°F (28° to 32°C) Severe: < 82°F (28°C) - The severely hypothermic patient may appear to be clinically dead, keep working until patient is warm > 90°F - Immediately Transport for rewarming at hospital	Mild: 93° to 95°F (34° to 35°C) Moderate: 86° to 93°F (30° to 34°C) Severe: < 86°F (30°C) - The severely hypothermic patient may appear to be clinically dead, keep working until patient is warm > 95°F - Immediately Transport for rewarming at hospital
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Blood Sugar Check - For patients who have ALOC.	
Warming Measures	
Remove wet clothing. If the patient requires movement, be gentle. Avoid unnecessary movement or rubbing. Cover patient with a blanket to prevent heat loss, keep patient horizontal.	
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG – Apply Defibrillation Pads as appropriate. Vascular Access - Most severely hypothermic patients are volume depleted – Administer warm IV fluids, Refer to EMS Policy #738 – Non-Traumatic Shock.	
CPR	
If patients are pulseless and apneic, initiate CPR. Limit defibrillation of V-fib/V-tach to one shock Once temperature > 86°F (30°C), defibrillate V-fib/V-tach as necessary Withhold IV medication until the temperature > 86°F (30°C). Refer to EMS Policy #702 – Cardiac Arrest.	

Signatures on File

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